

**State of Vermont
Secretary of State
Office of Professional Regulation
Board of Allied Mental Health Practitioners**

In re: Karen Giroux

License No. 068.0135315

Docket Nos. 2026-56 & 2026-57

STIPULATION AND CONSENT ORDER

STIPULATION

NOW COMES the State of Vermont, by and through Prosecuting Attorney Zoe Newman, and Respondent Karen Giroux, by and through her counsel Brice Simon, Esq., who hereby stipulate and agree as follows:

Board Authority

1. The Vermont State Board of Allied Mental Health Practitioners (Board) has authority to issue warnings or reprimands; suspend, revoke, limit, or condition current licenses or registrations; or prevent the renewal of lapsed licenses or registrations if, after disciplinary hearing, the Board finds that the Respondent has engaged in unprofessional conduct. 3 V.S.A. §129(a); 3 V.S.A. §129a; 3 V.S.A. §814; 26 V.S.A. Chapter 65; the Administrative Rules of the Board of Allied Mental Health Practitioners; and the Rules of the Office of Professional Regulation (OPR).

Stipulated Facts

2. Karen Giroux (Respondent) of Alburgh, Vermont, is licensed by the State of Vermont as a Licensed Clinical Mental Health Counselor (LCMHC) under license number 068.0135315. This license was first issued on June 21, 2023, and expires on January 31, 2027.
3. Respondent is additionally licensed by the State of Vermont as a Licensed Alcohol and Drug Abuse Counselor (LADAC) under license number 151.0134124. This license was first issued on September 30, 2021, and expires on January 31, 2027.
4. At all relevant times, Respondent worked as a private practice mental health practitioner, providing psychotherapy to clients in Vermont both in person, in St. Albans, and through telehealth.

HEALTH AND FITNESS

5. In February 2024, Respondent began to feel "not like herself."

6. In September 2024, Respondent began to suffer from acute memory loss, seizures, deliriums, and psychosis.
7. Respondent explained that she also started to "lose time."
8. Respondent attributed the symptoms listed in paragraphs six through eight to a severe bacterial infection.
9. Respondent believed these symptoms made her act in a way that was inconsistent with who she was when she was healthy.
10. In January or February of 2025, in a violent episode, Respondent repeatedly hit and spit on her then-romantic partner while screaming.
11. According to Respondent, her behavior during the fight was a result of her psychosis.
12. On an unknown date, Respondent and the same romantic partner had another fight.
13. During the second fight, Respondent held a gun while yelling.
14. Respondent ignored her romantic partner's requests to put the gun down.
15. On or about December 2, 2025, Respondent fired a gun into her kitchen floor after she thought she heard a noise in her house.
16. Despite the mentally altering medical symptoms Respondent has suffered since September 2024, she has continued to see psychotherapy clients.
17. Respondent stated that she is relying on her work to keep her grounded and not "floating in and out of confusion and delirium."

DUAL RELATIONSHIPS

18. On or about October 9, 2025, Respondent began providing psychotherapy to a client, herein referred to as E.B, on a reoccurring basis.
19. E.B. was in her early twenties.
20. In January 2026, E.B. was unhoused, along with her toddler and cat.
21. In January 2026, Respondent's daughter was staying with Respondent during her college break.
22. According to Respondent, Respondent's daughter offered to take care of E.B.'s cat and Respondent learned of this arrangement when she saw E.B.'s car in Respondent's driveway.

23. The following day, Respondent contacted E.B. to terminate their therapeutic relationship.
24. On December 30, 2025, Respondent held her last psychotherapy session with E.B.
25. Sometime between December 30, 2025, and January 18, 2026, and upon Respondent's offer, E.B. and her toddler moved into Respondent's home.
26. When asked by OPR Investigator K.B. to provide further details regarding Respondent's offer for E.B. to move in, Respondent stated she could not remember due to the symptoms described in paragraphs six through eight.
27. In late February 2026, Respondent, Respondent's daughter, and E.B. went on a two-night trip to Lake George, New York.
28. According to Respondent, she offered to pay for E.B. to restart psychotherapy with a new practitioner.
29. When speaking with Investigator K.B., Respondent denied that she had formed a friendship with E.B., instead calling their relationship an "unavoidable interpersonal connection."
30. Respondent further stated that her relationship with E.B. could not be an equal friendship due to the power differential inherent in their age gap as Respondent is approximately twice E.B.'s age.

CLINICAL DOCUMENTATION AND BILLING

31. As of March 16, 2026, Respondent was behind on sixty clinical notes as well as general administration for her psychotherapy practice.
32. Included in the administrative tasks Respondent needed to complete was correcting errors in her clinical and billing documentation.
33. Respondent told Investigator K.B. that she had initially become confused and completed E.B.'s intake paperwork under a different client's name.
34. Respondent also stated that she had billed E.B.'s insurance for another client's appointment after terminating E.B.'s psychotherapy, again, having become confused between clients.
35. Respondent attributed the errors as well as being behind on clinical notes and administrative documentation to her medical symptoms discussed in paragraphs six through eight.
36. Respondent further stated she had to double-check clinical records to make sure she completed them.

37. When discussing her medical symptoms and ability to recall events, including her clinical documentation, Respondent told Investigator K.B.: "I cannot justify anything 100% right now."
38. On April 16, 2026, the Board and an Administrative Law Officer held a consolidated contested hearing on the Request to Summarily Suspend Respondent's LCMHC license and Respondent's LADAC license, respectively.
39. On April 20, 2026, the Board and an Administrative Law Officer each issued written orders indefinitely suspending Respondent's LCMHC and LADAC licenses.

Violation One: 3 V.S.A. § 129(a)(5): Practicing the profession when medically or psychologically unfit to do so.

40. Paragraphs 2 through 39 above are incorporated herein.
41. Vermont law prohibits LCMHCs from practicing the profession when medically or psychologically unfit to do so.
42. Paragraphs 2 through 39 demonstrate that Respondent practiced licensed clinical mental health counseling when medically or psychologically unfit to do so when Respondent continued to see psychotherapy clients while also experiencing acute memory loss, deliriums, "losing time," and psychosis.
43. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because Respondent committed unprofessional conduct in violation of 3 V.S.A. § 129(a)(5).

Violation Two: 3 V.S.A. §129a(a)(28): Engaging in conduct of a character likely to deceive, defraud, or harm the public.

44. Paragraphs 2 through 39 above are incorporated herein.
45. Vermont law prohibits LCMHCs from engaging in conduct of a character likely to deceive, defraud, or harm the public.
46. Paragraphs 2 through 39 demonstrate that Respondent engaged in conduct likely to deceive, defraud, or harm the public when Respondent: 1. continued to provide psychotherapy to clients while experiencing acute memory loss, deliriums, "losing time," and psychosis; 2. billed client E.B.'s insurance for a different client's psychotherapy appointment; 3. invited client E.B. to move into Respondent's home within weeks of terminating the counseling relationship, and; 4. hit and spit on her romantic partner on one occasion, and held a gun while yelling at him on another occasion.
47. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because Respondent committed unprofessional conduct in violation of 3 V.S.A. §129a(a)(28).

Violation Three: 3 V.S.A. §129a(a)(15): Failing to exercise independent professional judgment in the performance of licensed activities when that judgment is necessary to avoid action repugnant to the obligations of the profession.

48. Paragraphs 2 through 39 above are incorporated herein.

49. Vermont law requires LCMHCs to exercise independent professional judgment in the performance of licensed activities when that judgment is necessary to avoid action repugnant to the obligations of the profession.

50. Paragraphs 3 through 39 demonstrate that Respondent failed to exercise independent professional judgment in the performance of licensed activities when Respondent: 1. continued to provide psychotherapy to clients while experiencing acute memory loss, deliriums, "losing time," and psychosis; 2. billed client E.B.'s insurance for a different client's psychotherapy appointment, and; 3. invited client E.B. and E.B.'s toddler to move into Respondent's home within weeks of terminating the counseling relationship.

51. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because Respondent committed unprofessional conduct in violation of 3 V.S.A. §129a(a)(15).

Violation Four: 3 V.S.A. § 129a(b)(2): Failure to practice competently by reason of any cause on a single occasion or on multiple occasions may constitute unprofessional conduct, whether actual injury to a client, patient, or customer has occurred. Failure to practice competently includes: failure to conform to the essential standards of acceptable and prevailing practice.

52. Paragraphs 2 through 39 above are incorporated herein.

53. Vermont law requires LCMHCs to practice competently by conforming to the essential standards of acceptable and prevailing practice.

54. Under the Vermont Administrative Rules for Allied Mental Health Practitioners, Rule 6.6, failure to comply with the American Counseling Association's (ACA) Code of Ethics and Standards of Practice may constitute unprofessional conduct.

55. The ACA's Code of Ethics, Rule A.6.e. states: "Counselors avoid entering into non-professional relationships with former clients, . . . or their family members when the interaction is potentially harmful to the client."

56. Paragraphs 2 through 39 demonstrate that Respondent entered into a non-professional relationship with a former client and the former client's family member when Respondent, all within a few months of terminating the therapeutic relationship: 1. invited client E.B. and E.B.'s toddler to move into Respondent's home; 2. when Respondent went on a trip with E.B., and; 3. when Respondent offered to pay for E.B.'s future psychotherapy appointments when these acts were potentially harmful to E.B. due to the power differential between Respondent and E.B. and that E.B. was now reliant on Respondent for housing.

57. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because Respondent committed unprofessional conduct in violation of 3 V.S.A. § 129a(b)(2).

Understandings

58. Respondent admits the facts above are true and the conditions below are necessary to protect the public.
59. Respondent understands that the Board must review and accept the terms of the Consent Order. If the Board rejects any portion, the entire Stipulation and Consent Order shall be null and void.
60. Respondent specifically waives any claim that any disclosures made to the Board during review of this agreement have prejudiced Respondent's right to a fair and impartial hearing if this agreement is not accepted and proceeds to a hearing on the merits before the same Board.
61. Respondent has read and reviewed this entire document and agrees it contains the entire agreement between the parties.
62. Respondent is not under the influence of any drugs or alcohol at the time of signing this Stipulation and Consent Order.
63. Respondent voluntarily enters into this agreement after the opportunity to consult with legal counsel and is not being coerced by anyone into signing this Stipulation and Consent Order.
64. Respondent voluntarily waives the right to a contested hearing before the Board and waives any right to appeal from this Stipulation and Consent Order.
65. Respondent understands that she is responsible for all compliance costs associated with this Stipulation and Consent Order.
66. Respondent agrees the Board may enter the Order set forth below.

Order

Based on the Stipulation above, it is **ORDERED AND ADJUDGED** as follows:

- A. The Board of Allied Mental Health Practitioners hereby **INDEFINITELY SUSPENDS** Respondent's LCMHC license for a **MINIMUM of ONE YEAR** beginning on the date of entry. Respondent must petition for reinstatement but may not do so for a period of one year. No sooner than forty-five (45) days prior to requesting reinstatement, Respondent must, at her own cost:
1. Obtain an independent neuropsychological evaluation report by a licensed PhD level Psychologist, whose license is in good standing, that the Enforcement Unit Case Manager (Case Manager) pre-approves (Independent Evaluator). The Independent Evaluator must verify receipt

of this Stipulation and Consent Order. The Independent Evaluator must also verify that he or she has no personal or professional background or history with Respondent. The neuropsychological evaluation report shall meet the independent evaluation requirements posted on OPR's website. The results of the neuropsychological evaluation report must indicate that Respondent is fit to practice and does not present a risk to the safety, health, or welfare of Respondent's potential patients/clients, as well as any recommendations that would ensure Respondent remains safe to practice. The independent neuropsychological evaluation report must be submitted to the Case Manager.

2. If the Independent Evaluator recommends, enter into a treating professional agreement and treatment plan with a treating professional(s) that the Case Manager approves and follow all recommendations. If treatment is recommended, Respondent must submit a treating professional agreement to the Case Manager before a reinstatement request will be considered.
- ii. If reinstated, Respondent's license **SHALL BE CONDITIONED** pursuant to whatever terms are reasonable at the time of Respondent's reinstatement request, taking into account the independent neuropsychological evaluation report, this Stipulation and Consent Order, and any other relevant factors at that time. Respondent's license may also be warned or reprimanded if reinstated.
- B. Pursuant to 3 V.S.A. § 125(b)(11), a Disciplinary Action Surcharge in the amount of two-hundred and fifty dollars (\$250.00) is assessed against Respondent. Respondent shall remit payment within six (6) months of the date of entry of this Order. Payment of the surcharge may be on a payment plan if needed. Failure to remit payment may result in the non-renewal of Respondent's license.
- C. Notwithstanding any provision above, Respondent must continue to meet all requirements for maintaining a license, license renewal and license reinstatement.
- D. This Stipulation and Consent Order is a matter of public record and may be reported to other licensing authorities as provided in 3 V.S.A. § 129(a).
- E. This Stipulation and Consent Order applies to all licenses this Board issues to Respondent, including future licenses, as provided in 3 V.S.A. § 129(l).
- F. This Stipulation and Consent Order will remain part of Respondent's licensing file and may be used for purposes of determining sanctions in any future disciplinary matter.

AGREED TO:

**STATE OF VERMONT
SECRETARY OF STATE**



Dated: 6/10/2026

Signed by:

Zoe Newman

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Zoe Newman
Prosecuting Attorney

Respondent

Signed by:

Karen Giroux

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Karen Giroux

Counsel for Respondent

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Brice Simon

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Brice Simon

APPROVED AND SO ORDERED:

**Vermont Board of Allied Mental
Health Practitioners**

DocuSigned by:

Tammy Austin

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Chairperson

Dated: 6/18/2026

Date of Entry: 6/19/26